

1. Administrative & Study Identification

Full Study Title: Phase 3, multicenter, open-label study to assess the long-term safety and tolerability of rimegepant for the acute treatment of migraine (with or without aura) in children and adolescents ≥ 6 to < 18 years of age. **Short Title/Acronym:** Long-term Safety Study of Rimegepant in Pediatric Subjects for the Acute Treatment of Migraine **Protocol Number:** C4951003 (Other IDs: BHV3000-312, 2024-512744-32-00) **NCT Number:** NCT04743141 **Posting ID:** 929267991520419160 **Sponsor Name:** Pfizer (Company: Pfizer) **Investigational Product:** Rimegepant (Trade Name: Nurtec | ATC Code: N02CD06 | Semantic Type: Pharmacologic Substance | Drug Preferred Name: rimegepant) **Phase of Development:** Phase III (Criteria: Trial must be in PHASE3) **Trial Status:** Recruiting (Currently recruiting participants) **Medical Monitor:** Pfizer Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the long-term safety and tolerability of rimegepant in children and adolescents (6 to < 18 years of age), focusing on the frequency and severity of on-treatment and treatment-emergent adverse events (TEAEs), serious adverse events (SAEs), adverse events leading to discontinuation, and clinically significant lab abnormalities. **Secondary Objectives:** To evaluate the frequency and severity of hepatic-related adverse events and the frequency of hepatic-related treatment discontinuations.

2.2 Background & Rationale Migraine is a debilitating neurological condition affecting a significant portion of the pediatric population. The study addresses the medical necessity of providing safe, effective acute treatments for children and adolescents. Rimegepant (a CGRP receptor antagonist) has established efficacy in adults, and this long-term, open-label extension study is conducted to characterize its long-term safety, tolerability, and hepatic profile when used as an acute treatment in pediatric patients weighing > 15 kg.

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm **Blinding:** Open-label **Randomization:** N/A (Open-label extension/Long-term Safety)

3.2 Planned Enrollment & Duration Target **\$N\$:** 600 participants
Number of Sites: Multicenter (Global, including US and Europe)
Estimated Study Start: 27-Apr-2021 **Estimated Primary Completion:** 30-Jul-2030

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. History of migraine (with or without aura) for ≥ 6 months before Screening. 2. History of 1 to 8 moderate or severe attacks per month during the 2 months prior to screening. 3. 1 or more migraine days requiring treatment during the Observation Phase. 4. Prophylactic migraine medication is permitted if the dose has been stable for at least 12 weeks prior to the Screening Visit. 5. Ability to distinguish between migraine and other types of headaches. 6. Weight > 15 kg (for EU countries: Participants 12 to < 18 years of age must have a body weight of > 25 kg). 7. Adequate venous access for blood sampling. 8. Male and female participants 6 to < 18 years of age (participants must not reach their 18th birthday on or before the Baseline visit).

4.2 Exclusion Criteria 1. History of cluster headache or hemiplegic migraine headache. 2. Confounding and clinically significant pain syndrome. 3. Current uncontrolled and/or untreated psychiatric condition for a minimum of 6 months prior to the Screening Visit (lifetime history of psychosis and/or mania are excluded). 4. History of suicidal behavior or at risk of self-harm/harm to others. 5. History of major psychiatric disorder. 6. Current diagnosis or history of substance abuse. 7. Reported current use of or tested positive at Screening for drugs of abuse.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Dosage/Frequency: Rimegepant administered acutely as needed for migraine attacks. Dosing is weight-based (e.g., 35 mg, 50 mg, or 75 mg). **Route of Administration:** Oral (Orally disintegrating tablets). **Duration of Treatment:** Long-term safety follow-up.

5.2 Concomitant & Prohibited Therapy Permitted: Prophylactic migraine medication is permitted if the dose has been stable for at least 12 weeks prior to the Screening Visit. **Prohibited:** Concomitant use of prohibited acute medications or CYP3A4 strong inhibitors/inducers as per protocol guidelines.

6. Study Timeline & Visit Schedule

Visit ID	Window (Days)	Key Activities/Procedures	:---
:---	:---	Screening	Day -28 to 0 Informed Consent/

Assent, Medical History, Weight, ECG, Labs | | **Observation/
Baseline** | Day 0 | Confirmation of eligibility, First Dose
Dispensation, Electronic Diary setup | | **Interim Follow-ups** |
Every 4-12 Weeks | Safety Labs, Hepatic Monitoring, Efficacy
Assessment, IP Accountability | | **End of Study** | Study Completion
| Final Vital Signs, Exit Interview, IP Accountability, Final
Safety Labs |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Definition: The frequency and severity of on-treatment and treatment-emergent adverse events (TEAEs), serious adverse events (SAEs), adverse events leading to discontinuation, and clinically significant laboratory abnormalities. **Measurement Method:** Clinical assessment, physical examinations, and Central Laboratory Analysis.

7.2 Secondary & Exploratory Endpoints **Endpoint 1:** Frequency and severity of hepatic-related adverse events. **Endpoint 2:** Frequency of hepatic-related adverse events leading to treatment discontinuation.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Standard collection via patient/parent electronic diary, HCP interviews during site visits, and routine vital/laboratory assessments. **Serious Adverse Events (SAEs):** Standard reporting timeline; specifically monitoring for severe hepatic events or systemic reactions. **Data Monitoring Committee (DMC):** Internal/External independent committee established by the sponsor to review long-term pediatric safety data.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Safety Set (all patients who received at least one dose of the investigational product). **Sample Size Justification:** A sample size of $N=600$ is planned to provide sufficient exposure data for the robust characterization of the long-term safety and tolerability profile of rimegepant in the pediatric population. **Handling of Missing Data:** Standard conventions for safety data, with no imputation for missing adverse events.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring visits to ensure protocol adherence and safety tracking. **Audit Trail:** Details on eCRF locking and electronic patient diary data clarification forms.

11. Study Identifiers & Governance

Primary ID: C4951003 **Registry Number:** NCT04743141 **Posting ID:** 929267991520419160 **Sponsor/Lead Organization:** Pfizer **Study Title:** Long-term Safety Study of Rimegepant in Pediatric Subjects for the Acute Treatment of Migraine **Official Regulatory Title:** Phase 3, multicenter, open-label study to assess the long-term safety and tolerability of rimegepant for the acute treatment of migraine (with or without aura) in children and adolescents 6\$ to \$< 18\$ years of age.

12. Clinical Framework (Migraine Specific)

Disease Area: Acute Treatment of Migraine **Condition Name:** Acute Treatment of Migraine **Preferred UMLS Name:** Acute Migraine Treatment **Preferred Name:** Drugs used for the treatment of acute migraine **Definition:** Acute Treatment of Migraine refers to the use of medications or other interventions to stop or relieve an active migraine attack once it has begun. The primary goal is to alleviate pain and associated symptoms like nausea, photophobia, and phonophobia, thereby restoring function and improving the patient's quality of life during an episode. This approach is distinct from preventive treatments and typically involves medications such as non-steroidal anti-inflammatory drugs (NSAIDs), triptans, ditans, or gepants, chosen based on the severity of the attack and individual patient factors. **Diagnosis Threshold:** 1 to 8 moderate or severe attacks per month **Medical Methodology:** Acute treatment (CGRP receptor antagonist) **SNOMED ID:** 710438008 **SNOMED Hierarchy:** 446700008 | Treatment for headache | Treatment for headache (regime/therapy) **MeSH Code:** N/A **Optimization Target:** Safe, long-term PRN (as needed) treatment without clinically significant hepatic/laboratory abnormalities.

13. Study Procedures & Methodology

Management Pathway: Standard acute clinical pathway for pediatric migraine **Education Protocol (HCP):** Training on protocol guidelines, weight-based dosing requirements, and AE reporting criteria. **Education Protocol (Patient):** Caregiver and patient instruction on properly utilizing electronic diaries and distinguishing between migraine and other types of headaches. **Quality Audit Mechanism:** Safety parameter tracking, caregiver compliance monitoring, and eDiary adherence.

14. Observation & Follow-Up Schedule

Total Duration: Estimated 2021 to 2030 (Long-term extension)

Onsite Visit Cadence: Regular intervals (e.g., Weeks 4, 12, 24, and every 12 weeks thereafter) **Remote Monitoring Frequency:**

Regular safety calls as per protocol **Checklist Review Interval:**

Routine eDiary review at each visit

15. Sign-off & Approval

Role	Name	Signature	Date		----	----	----	----
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]					
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]					Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]					